



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : جيهان فهم عبدالرحيم حسنين
المحافظة : القاهرة
الحى / المدينة : قصر النيل
رقم البطاقة : 26608020100784 رقم التليفون : 01121113144
العمر (تاريخ الميلاد) : 02-08-1966 المهنة : ربه منزل
العنوان : --
تاريخ تسجيل الإستمارة : 28-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي تراستوزوماب تحت الجلد و
بيرتوزوماب (فيسجو) [14117]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: جيهان فهم عبدالرحيم حسنين National ID: 26608020100784	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي (Phesgo (Subcutaneous trastuzumab and pertuzumab

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
مطابقة تحقيق الشخصية

جوشن

فهييم عهدا الرحيم حمتين

١١ ش الكامل محمد الزمك

قصر النيل - القاهرة

١٩٩٦/٠٨/٢٧

٢٦٦٠٨٠٢٠١٠٠٧٨٤

J10208708

-١١٤١١١٤١٤٤



٢٠٢٩/٠٣

٢٦٦٠٨٠٢٠١٠٠٧٨٤

ربة منزل

ارطلة

مصلحة

التي



البيطافه منارية حتى ٢٠٢٩/٠٣/١٥





معامل الخلية

للخبايا الأنسجة والأورام

Patient Name	جهان فهم عبدالرحيم	Code Number	YK 26024955
Sex/Age	Female 60 Y 0 M 0 D	Patient ID	101650
Registered Date	06/8/2026 07:06:59 PM	Referred By Dr.	عمرو الطويطي
Validation Date	11/8/2026 03:37:11 PM	Branch	الفرع الرئيسي للتقى
Reporting Date	12/8/2026 01:16:16 PM		

Clinical Diagnosis: Right Breast Suspicious Mass Lesion
Nature of the specimen: U/S Guided TCNB

Pathology Report

Gross Picture

Right breast suspicious looking mass lesion from where U/S guided tru cut biopsy taken. Few cores provided for histopathological assessment.

Multiple grey, white tissue cores measured 7.3 cm in total length, totally submitted.

Biopsied by: Dr.Khaled Elsaied.

Microscopic Picture

Sections of breast core biopsy show infiltrative malignant epithelial proliferation arranged in irregular nests, cords, and tubules within a desmoplastic stromal background. The tumor cells exhibit moderate nuclear pleomorphism and mitotic activity. Moderate stromal tumor-infiltrating lymphocytic response is present. Foci of associated ductal carcinoma in situ (DCIS), intermediate nuclear grade, are identified, comprising approximately 5% of the submitted core tissue. No definite peritumoral lymphovascular or perineural invasion is identified in the examined material.

Diagnosis

Right Breast Suspicious Mass Lesion, U/S Guided TCNB:

Invasive Breast Cancer Of NST (Invasive Duct Carcinoma) Grade II (B5).

Moderate Tumor Infiltrating Lymphocytes.

Associated Ductal Carcinoma In Situ Components Of Intermediate Grade, Representing 5% Of Submitted Core Tissue.

No Evident Peritumoral Lymphovascular Permeation Or Perineural Invasion In Current Tissue.

For Clinico-Radiologic Correlation,

Comments:

-ER, PR, Her2-nen And Ki-67 Are Available Upon Request.

- Royal collage of pathologists breast lesions reporting categories:-

B1. Normal tissue.

B2. Benign lesion.

B3. Lesion of uncertain malignant potential.

B4. Suspicious lesions.

B5. Malignant lesions.

P.O

Number of slides enclosed: 2

Diagnosed by

Consultant Pathologist

Dr. Mona Sakr, MD

Scan Me



الفرع الرئيسي الدقي : ٢٠ شارع التحرير أمام مترو البحوث - الدقي - الدور الأول
م: ٠١١١٢٤٤٣٣٣ - ٠١١١٥٥٧١١٧ - ت: ٠٢٣٣٣٥٣٠٧٣

Name: جيهان فهم عبدالرحيم حسنين			
MR. : 13718			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 60-year-old postmenopausal individual with a negative family history and no reported comorbidities. The condition began two months ago when an incidental right breast mass was discovered; the patient had previously had a normal mammosonography. Clinical examination revealed a right breast lump measuring approximately 5 × 4 cm and a right axillary lymph node measuring 2 × 2 cm. Core needle biopsy demonstrated invasive ductal carcinoma, grade II, with immunohistochemistry showing estrogen receptor 0/8, progesterone receptor 8/8, HER2 3+, and Ki-67 not specified. Bilateral breast ultrasound identified a right-sided lesion measuring 3.7 × 3.3 × 2 cm and two suspicious right axillary lymph nodes. Staging PET/CT scan showed no distant metastases. The referring team recommended revision of the immunohistochemistry, bilateral mammography, and then consideration of neoadjuvant chemotherapy (NACT) with THP. The multidisciplinary team concurred with the need for IHC revision and bilateral mammography, proceeded with portacath insertion, and planned NACT with Taxol/Phesgo for six cycles, followed by consideration of right-sided modified radical mastectomy.			
تحريراً في ,			
		التاريخ :	23-09-2026
		الوقت :	
		الطبيب المعالج :	
يعتمد ,			

Name:	Geziane Fahim Abdalrhim Hassanin	Age :	59.7
Referred by:	Prof. Dr/ ...	Date :	06/08/2026
Examination:	US- Breast		

BILATERAL BREAST ULTRASONOGRAPHY

- The right breast show irregular hypo echoic solid lesion at lower outer quadrant measures 3.7x3.3x2 cm showing internal echogenic dots of micro calcifications as well as internal vascularity , associated with two suspicious right axillary lymph nodes , one is rounded shape measure 11x9mm , another one show loss of hilum measures 4x2.5cm Sonographic feature of suspicious breast lesion (BIRADS V) For FNAC .
- Both breasts show few simple cysts , largest seen left breast measures 6x5mm at 1 o'clock , some of them at left breast show internal echogenic foci .
- Normal sonographic appearance of breast nipples, areola complex with no retro-areolar masses.
- Normal skin thickness, with normal subcutaneous structures.
- Normal caliber of lactiferous ducts with no ectatic changes.
- No left suspicious axillary lymph nodes .

OPINION:

- ✓ Right breast suspicious lesion with suspicious two right axillary lymph nodes for FNAC
- ✓ Bilateral fibro cystic changes of both breasts .

Best regards,
 Dr. Heba Mohamed



رسوم الاستعلام 20 جنية

جهة الاصدار : فرع القاهرة

تاريخ الاصدار : 8-9-2026

الى من يهمه الامر

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للانتفاع الخاضعين لقانون التأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "القوى العاملة والمعاشات والارامل " و كذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012 " الاطفال دون السن المدرسي " وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة .
وطبقا لقاعدة البيانات المرسله من التأمينات الاجتماعيه
وطبقا لقاعدة البيانات المرسله من الضمان الاجتماعي
والبحث تبين أن

السيد (ة) جيهان فهد عبد الرحيم حسنين بطاقة رقم 26608020100784

تليفون رقم: لا يوجد

وطبقا الى خطاب من التأمينات الاجتماعية يفيد عدم الانتفاع
بمميزات غير المستفيعه بالتأمين الصحي صدر هذا الخطاب دون اننى مسئولية على الهيئة العامة للتأمين الصحي حال خضوع المواطن لقانون التأمين الصحي الكامل

تاريخ الصياغة : 8-9-2026

الموظف المختص

محمود عبدالله



اقادة مركز نظم المعلومات
يعتمد،،،

مسلسل 2073430



الخطاب ساري لمدة ثلاثة اشهر من تاريخ الاصدار الا في حالة تغير حاله المواطن من قبل الهيئة القومية للتأمينات الاجتماعية او وزارة التضامن الاجتماعي او وزارة التربية والتعليم

الإدارة المركزية لنظم المعلومات و التحول الرقمي

Medical Oncology	Surgery	Radiology	Pathology
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SIGNATURE

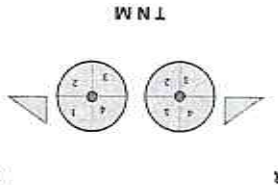
Recommendations:
 For: 1. Revision of IHC 2. Bilateral Mammography
 Then consider NACT THP
 MDT Outcome & Plan
 For:
 1. Revision of IHC
 2. Bilateral Mammography
 3. Portacath insertion... Then consider NACT Taxol/Phesgo x 6 cycles... then consider Rt MRM.

Clinical Staging:							
CT4b N2 M0							
Pathological Staging:							
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4
Right Side				Left Side			

Mets Workup:
 PET/CT scan... No distant mets
 Biomarker/Molecular
 Bilateral breast US... Rt breast LOQ mass 3.7 x 3.3 x 2 cm, with two
 suspicious Rt AxLNs
 Sonomammography:
 O/e... Rt breast LOQ mass 5x4 cm with Rt AxLN 2x2 cm

History:
 The condition started 2 mths ago, by accidental discovery of a Rt breast mass, that the pt neglected as her previous mammosonography was free... mammosonography & biopsy revealed IDC GII, ER0/8 PR8/8 Her2 3+... PET/CT scan no mets

ER: PR: HER2: KI67:



Clinical Data:

Name:	MRN:	13718
Age:	ID:	26608020100784
Menopausal Sta	Phone:	01121113144
NRS SCORE:	Marital Status:	
BMI:	Family History:	-ve
CO Morbidity:	Reason OF MDT:	
Surgical History:	ECOG Performance Status	0 1 2 3 4

MDT FORM

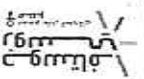
09/07/2026



منظمة الصحة العالمية
 المملكة العربية السعودية



9-11



Best regards,
Dr. Heba Mohamed

- ✓ Right breast suspicious lesion with suspicious two right axillary lymph nodes for FNAC
- ✓ Bilateral fibro cystic changes of both breasts .

OPINION:

- The right breast show irregular hypo echoic solid lesion at lower outer quadrant measures 3.7x3.3x2 cm showing internal echogenic dots of micro calcifications as well as internal vascularity , associated with two suspicious right axillary lymph nodes , one is rounded shape measure 1x9mm , another one show loss of hilum measures 4x2.5cm Sonographic feature of suspicious breast lesion (BIRADS V) For FNAC .
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- Normal caliber of lactiferous ducts with no ectatic changes.
- No left suspicious axillary lymph nodes .

BILATERAL BREAST ULTRASONOGRAPHY

Name:	Gehane Fahim Abdalrhim Hassanin		
Referred by:	Prof. Dr / ...1		
Examination:	US- Breast		
	Age :	Date :	06/08/2026
	59.7		



Pat. Name: Jihan Fahim Abd Al RahimPat. ID: 3636279
Procedure Name: PET - CT, NuclearDate: 30/08/2026

F18-FDG PET/CT STUDY

Clinical History:

- 61 years old patient with right breast cancer.
- PET/CT requested for initial assessment.

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCI was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV, a Semi-quantitative analysis would be done for selected ROIs and the normal threshold is < 2.5 .
- Hepatic reference activity: 3.8

Report:

No previous PET-CT available for comparison.



Head and Neck:

- There is no hypermetabolic cervical lymphadenopathy.
- Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- The brain shows uniform tracer distribution with NO hypermetabolic focal lesions.

Chest:

- Right breast FDG avid retro-areolar and LOQ il-defined neoplastic lesion; measuring roughly about 2.9x2.2cm with SUVmax~12.3.



Diagnosed by

Number of slides enclosed: 2

P.O.

B1. Normal tissue.
B2. Benign lesion.
B3. Lesion of uncertain malignant potential.
B4. Suspicious lesions.
B5. Malignant lesions.

Comments: ER, PR, Her2-neu And Ki-67 Are Available Upon Request.
-Royal collage of pathologists breast lesions reporting categories:-
Normal tissue

Right Breast Suspicious Mass Lesion, U/S Guided ICB:
Invasive Breast Cancer Of NST (Invasive Duct Carcinoma) Grade II (B5).
Moderate Tumor Infiltrating Lymphocytes.
Associated Ductal Carcinoma In Situ Components Of Intermediate Grade, Representing 5% Of Submitted Core Tissue.
No Evident Peritumoral Lymphovascular Permeation Or Perineural Invasion In Current Tissue.
For Clinico-Radiologic Correlation.
Comment:
RP BR Her2-neu And Ki-67 Are Available Upon Request.

Diagnosis

Sections of breast core biopsy show infiltrative malignant epithelial proliferation arranged in irregular nests, cords, and tubules within a desmoplastic stromal background. The tumor cells exhibit moderate nuclear pleomorphism and mitotic activity. Moderate stromal tumor-infiltrating lymphocytic response is present. Foci of associated ductal carcinoma in situ (DCIS), intermediate nuclear grade, are identified, comprising approximately 5% of the submitted core tissue. No definite peritumoral lymphovascular or perineural invasion is identified in the examined material.

Microscopic Picture

assessment.
Multiple grey, white tissue cores measured 7.3 cm in total length, totally submitted.
Biopsied by: Dr.Khaled Elsaid.

Gross Picture

Gross Picture

Right breast suspicious looking mass lesion from where U/S guided tru cut biopsy taken. Few cores provided for histopathological

Pathology Report

Clinical Diagnosis: Right Breast Suspicious Mass Lesion
Nature of the specimen: U/S Guided TCNB

Patient Name	فهد بن محمد بن محمد
Sex/Age	Female 60 Y 0 M 0 D
Registered Date	06/8/2026 07:06:59 PM
Validation Date	11/8/2026 03:37:11 PM
Reporting Date	12/8/2026 01:16:16 PM
Code Number	1011650
Referred By Dr.	Dr. محمد بن محمد بن محمد
Patient ID	1011650
Branch	العيادة العامة
YK 26024955	1011650



၂၀၁၆ ခုနှစ် ဇူလိုင်လ ၁ ရက်နေ့
 နံနက် ၈ နာရီ ၀၀ မိနစ်



Scan Me

Dr. Mona Sakr, MD
Consultant Pathologist

Diagnosed By

Number of slides enclosed: 4

P.O

ER: Negative Expression (Score 0/8).
PR: Strong Positive (Score 8/8).
Her2-neu: Positive Overexpression (Score 3+).
Ki-67 Proliferation Index: About 35%.

Right Breast Suspicious Mass Lesion, Immunohistochemistry:

Diagnosis

-35% of tumor cells show positive nuclear staining for Ki-67. Positive external control is used.

-Sections treated for Her2-neu show strong complete membranous staining of in >10% tumor cells (Score 3+). Positive external control is used.

-Sections treated for ER show negative nuclear staining of tumor cells (Score 0/8). Positive external control is used.

Results

- ER
- PR
- Her2-neu
- Ki-67

Immunohistochemistry

Immunohistochemistry

Clinical Diagnosis: Right Breast Suspicious Mass Lesion
Nature of the specimen: Immunohistochemistry

Patient Name	جنات فهم عبدالرحمن	Code Number	YK 26027088
Sex/Age	Female 60 Y 0 M 17 D	Referred By Dr.	تامر النحاس
Registered Date	23/8/2026 10:25:38 PM	Branch	الفرع الرئيسي الطبي
Validation Date	30/8/2026 03:35:46 PM		
Reporting Date	31/8/2026 10:33:26 PM		



معمل الجزيئية
لتحليل الأنسجة والأورام

الفرع الرئيسي الطبي : شارع التحرير - الدور الأول
شارع التحرير - الدور الأول - الفرع الرئيسي : شارع التحرير - الدور الأول



Medical No. : 9928
Patient's Name: Gihan Fahim Abdulrahim Hasanen
Date: 2026-09-27
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Pathologically proved right breast carcinoma.

Findings:

Right breast.

- Extreme dense breast masking the underlying parenchyma (ACR D) .
- Right grouped suspicious microcalcifications in the right UOQ in the middle third
- **Ultrasound demonstrated the pathologically proved malignant mass is seen in the right breast at 8 o'clock associated with infiltrated parenchyma involving zones 1 ,2 and 3 measuring (4.8 x 2.4 cm).**
- Another right malignant retroareolar mass measuring (1.2 x 0.8 cm).
- Multiple (more than 5)pathological levels 1 ,2 and 3 right axillary LNs ,the largest measures 3.6 x 1.8 cm). No pathological supra clavicular lymph nodes.

Left breast.

- Extreme dense breast masking the underlying parenchyma (ACR D) .
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- FEW SCATTERED SUBCENTIMETERIC CYSTS .
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

- **Right breast (BIRADS 6): LOQ multi-focal malignant process with ipsilateral multiple pathological level 1& 2 ,3axillary lymph nodes.**
- **Left breast (BIRADS 2): Early fibrocystic changes**

Prof.MONA ZAKY

Patient Name	جيهان فهم عبدالحليم	Case No.	373607
Sex/Age	Female 60 Y	Patient No.	27825
Registered Date	09/9/2026 03:53:02 PM	Referred By Dr.	Out Patient
Validation Date	13/9/2026 03:33:43 PM	Branch	Gustave Roussy
Reporting Date	13/9/2026 03:41:35 PM	Pathology No.	SR-1040/26

Clinical Data: Right Breast Cancer.

Breast Slide Revision

Nature of Specimen: Referred Block & Slides (Tru-Cut Biopsy).

Gross Examination:

- Two referred paraffin blocks labelled by (26024955) and marked as #1 & #2. Together with, six referred slides including 2 (H&E) stained slides marked as #1 & #2 and 4 immunostained slides for (ER, PR, Her2, Ki67).

Microscopic Examination (of both blocks):

- * Histological type: Invasive duct carcinoma.
 - * Grade: 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
 - * Ductal carcinoma in situ: Absent.
 - * Angioinvasion: Present.
 - * Perineural invasion: Absent.
 - * Microcalcification: Present.
 - * Necrosis: Present.
 - * Tumor infiltrating lymphocytes (TILs): Moderate.
- IPT: (on both blocks)**
- ER: No nuclear reaction in tumor cells (score 0/8).
 - PR: Strong nuclear reaction in 50% of tumor cells (score 7/8).
 - Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
 - Ki67%: 40%.

Diagnosis:

Right Breast Mass; Referred Blocks & Slides (Tru-cut biopsy):

- Invasive Duct Carcinoma, Grade 3.

IPT:

- ER: Negative (0/8).

- PR: Positive (7/8).

- Her2: Positive (score 3+).

- Ki67%: 40%.

Patient Name	جيهان فهم عبدالحليم	Case No.	373607
Sex/Age	Female 60 Y	Patient No.	27825
Registered Date	09/9/2026 03:53:02 PM	Referred By Dr.	Out Patient
Validation Date	13/9/2026 03:33:43 PM	Branch	Gustave Roussy
Reporting Date	13/9/2026 03:41:35 PM	Pathology No.	SR-1040/26

* *Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* **Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.

د. سنان
روسي
رئيسة



ميسادة رئيسة الجمهورية
لدم صحة المرأة المصرية

الجمهورية
مصرية
الجديدة

09/07/2026

MDT FORM

Name:	سنان فهم علي محمد حنين	MRN:	13718
Age:	60 Y	ID:	26508020100784
Menopausal Sta	postmenopausal	Phone:*	01121113144
NRS SCORE:		Marital Status:	
BMI:		Family History:	-ve
Gender		Reason Of MDT:	New case
CO Morbidity:	-ve	ECOG Performance Status	0 0 1 0 2 0 3 0 4
Surgical History:*	-ve		

Clinical Data:

BX:

ER: PR: HER2: KI67:



TNM

History:

The condition started 2 mths ago, by accidental discovery of a Rt breast mass, that the pt neglected as her previous mammosonography was free...mammosonography & biopsy revealed IDC GII, ER0/8 PR8/8 Her2 3+..PET/CT scan no mets

O/e...Rt breast LOQ mass 5x4 cm with Rt AxLN 2x2 cm

Sonomammography:

Bilateral breast US...Rt breast LOQ mass 3.7 x 3.3 x 2 cm, with two suspicious Rt AxLNs

Biomarker/Molecular

Mets Workup:

PET/CT scan...No distant mets

Recommendations:

For: 1.Revision of IHC 2.Bilateral Mammography
Then consider NACT THP

MDT Outcome & Plan

For:

1.Revision of IHC
2.Bilateral Mammography
3.Portacath insertion...Then consider NACT Taxol/Phesgo x 6 cycles...then consider Rt MRN.

Clinical Staging:

CT4b N2 M0

Pathological Staging:

Q1 Right Side Q2 Q3 Q4 Q1 Left Side Q2 Q3 Q4

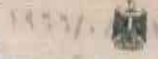
SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. سنان	د. محمد نور الدين		



جمهورية مصر العربية
بطاقة تحقيق الشخصية

جيهان
فهم عبد الرحيم حسنن
١١ ش الكامل محمد-الزمالك
قصر النيل - القاهرة



٢٦٦٠٨٠٢٠١٠٠٧٨٤

J10208708





الوزارة للصحة

Name:	Gehane Fahim Abdalrhim Hassaniin		Age :	59.7
Referred by:	Prof. Dr/ ... I		Date :	06/08/2026
Examination:	US- Breast			

BILATERAL BREAST ULTRASONOGRAPHY

- The right breast show irregular hypo echoic solid lesion at lower outer quadrant measures 3.7x3.3x2 cm showing internal echogenic dots of micro calcifications as well as internal vascularity , associated with two suspicious right axillary lymph nodes , one is rounded shape measure 1x9mm , another one show loss of hilum measures 4x2.5cm Sonographic feature of suspicious breast lesion (BIRADS V) For FNAC .
- Both breasts show few simple cysts , largest seen left breast measures 6x5mm at 1 o'clock , some of them at left breast show internal echogenic foci .
- Normal sonographic appearance of breast nipples, areola complex with no retro-areolar masses.
- Normal skin thickness, with normal subcutaneous structures.
- Normal caliber of lactiferous ducts with no ectatic changes.
- No left suspicious axillary lymph nodes .

OPINION:

- ✓ Right breast suspicious lesion with suspicious two right axillary lymph nodes for FNAC
- ✓ Bilateral fibro cystic changes of both breasts .

Best regards,

Dr. Heba Mohamed

 riadascanlab

٠١١٥٣٠٣٠٣٠٠٠ ٣

فرع إمبابة: ٧١ شارع الوحو حدة

Pat. Name: Jihan Fahim Abd Al RahimPat. ID: 3636279
Procedure Name: PET - CT. NuclearDate: 30/08/2026

F18-FDG PET/CT STUDY

Clinical History:

- o 61years old patient with right breast cancer.
- o PET/CT requested for initial assessment.

Technique:

- o For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROIs and the normal threshold is < 2.5.

- o Hepatic reference activity:3.8

Report:

No previous PET-CT available for comparison.



Head and Neck:

- * There is no hypermetabolic cervical lymphadenopathy.
- * Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- * The brain shows uniform tracer distribution with NO hypermetabolic focal lesions.

Chest:

- * Right breast FDG avid retro-areolar and LOQ il-defined neoplastic lesion; measuring roughly about 2.9x2.2cm with SUVmax~12.3.



معامل الخلية لتحليل الأنسجة والأورام

Patient Name جيهان فهم عبدالرحيم
Sex/Age Female 60 Y 0 M 17 D
Registered Date 23/8/2026 10:25:38 PM
Validation Date 30/8/2026 03:35:46 PM
Reporting Date 31/8/2026 10:33:26 PM

Code Number YK 26027088
Patient ID 101650
Referred By Dr. تامر التحاسن
Branch الفرع الرئيسي الدقي

Clinical Diagnosis: Right Breast Suspicious Mass Lesion
Nature of the specimen: Immunohistochemistry

Immunohistochemistry

Immunohistochemistry

- ER
- PR
- Her2-neu
- Ki-67

Results

-Sections treated for ER show negative nuclear staining of tumor cells (Score 0/8). Positive external control is used.
-Sections treated for PR show strong nuclear staining in 95% of tumor cells (Score 8/8). Positive external control is used.
-Sections treated for Her2-neu show strong complete membranous staining of in >10% tumor cells (Score 3+). Positive external control is used.
-35% of tumor cells show positive nuclear staining for Ki-67. Positive external control is used.

Diagnosis

Right Breast Suspicious Mass Lesion, Immunohistochemistry:

- # ER: Negative Expression (Score 0/8).
- # PR: Strong Positive (Score 8/8).
- # Her2-neu: Positive Overexpression (Score 3+).
- # Ki-67 Proliferation Index: About 35%.

P.O

Number of slides enclosed: 4

Diagnosed By

Consultant Pathologist
Dr. Mona Sakr, MD



Scan Me



الفرع الرئيسي الدقي : ٢ شارع التحرير أمام مترو الجوث - الدقي - الدور الأول
م: ٢٣٣٣٤٤٣٣ - ١١١١٢٤٤٣٣٣ - ١١١١٥٥٧١٧ - ٧٣ - ٢٣٣٣٥٣٣٥٣